

City Medical Center - Gastroenterology Department

1234 Elm Street, Suite 567, Metropolis, NY 12345

Phone: (555) 123-4567 | Fax: (555) 123-4568

Dr. John Doe, MD

Registration / Demographics

Name: John Smith

DOB: 04/12/1980

Age: 43

Sex: Male

Address: 123 Main Street, Metropolis,
NY 12345

Phone: (555) 987-6543

MRN: 123456789

Account: 987654321

Insurance: TestPlan Health

Emergency Contact: Jane Doe (555)
987-1234

Encounter Date: 03/15/2023

Check-in Time: 08:30 AM

Location: Endoscopy Suite

Attending Gastroenterologist/Endoscopist: Dr. John Doe, MD

Referring Provider: Dr. Emily Jones

Chief Complaint / Indication

Dyspepsia

Medical History

PMH: Hypertension, Diabetes

PSH: Appendectomy

FHX: Father - CAD

SHX: Non-smoker, Alcohol - social

Allergies: Penicillin

Problem List: GERD

Current Medications:

- Lisinopril 10 mg daily
- Metformin 500 mg BID
- Omeprazole 20 mg daily
- Aspirin 81 mg daily
- Vitamin D 1000 IU daily

Pre-procedure Vitals & ASA Classification

BP: 120/80 mmHg

HR: 75 bpm

SpO2: 98%

ASA: II

Informed consent for EGD obtained with risks and benefits discussed.

Pre-Procedure Assessment & Anesthesia Record	
Airway: Mallampati II	Cardio/Pulm: WNL
NPO Status: NPO after midnight	Last Intake: Water at 11:30 PM prior night
Planned Sedation: MAC with Propofol and Fentanyl	
Anesthesiologist: Dr. Anna Green, MD	CRNA: Paul Smith, CRNA
Procedure Times	
Anesthesia Start: 09:00 AM	Scope In: 09:15 AM
Scope Out: 09:45 AM	Anesthesia End: 10:00 AM
Recovery Start: 10:05 AM Recovery End: 10:35 AM	
Monitoring Trends	
BP: 110/70 -> 115/75 -> 112/74 mmHg	HR: 72 -> 80 -> 76 bpm
SpO2: 96% -> 98% -> 97%	Complications: Brief desaturation at 94%, resolved with O2 adjustment
Post-Anesthesia Evaluation	
Aldrete Score: 10	Disposition: Discharged home with escort

Procedure Note: Esophagogastroduodenoscopy (EGD)	
Endoscope Model: Olympus Exera III	Route: Oral
Patient Position: Left lateral decubitus	Bite Block: Yes
Procedure Narrative	
The endoscope was introduced via the oral cavity into the oropharynx, with smooth passage through the UES. Upon entering the esophagus, normal mucosal landmarks were visualized, with the Z-line noted at 40 cm from the incisors. The GEJ was visualized without stigmata of bleeding. Careful examination of the stomach found the fundus and body with intact rugal folds and no lesions. The antrum appeared slightly erythematous. Insufflation was used to distend the stomach. The pylorus was widely patent. Entering the duodenum, both the bulb and the second portion were examined, revealing normal villi architecture. There were no ulcers or significant inflammation observed. Retroflexion was performed in the stomach to assess the cardia and fundus, showing no abnormalities. The procedure was well tolerated, with no immediate complications.	
Findings	
- Normal esophageal mucosa - Mild erythema in gastric antrum - Normal duodenal bulb and second portion	
Impression	
Mild antral gastritis observed. No esophagitis or duodenal abnormalities.	
Diagnosis List	
Mild Gastritis, otherwise normal EGD.	
Interventions	
Cold forceps biopsy of the antrum taken for histological analysis.	
Endoscopy Team	
Endoscopist: Dr. John Doe, MD Assistant: Sarah Brown, RN RN Circulator: Lisa Carter, RN Endoscopy Tech: Mike Davis	

Specimens / Lab / Pathology	
Specimen: A	Site: Gastric Antrum
Number of Fragments: 2	Method: Cold forceps
Fixative: Formalin	Collection Time: 09:40 AM Sent to Lab: 09:50 AM
Lab Information	
Lab Name: City Pathology Services	Accession Number: LAB-TST-987654
Pathology Report	
Gross Description: Two small tan mucosal fragments. Microscopic Description: Gastric mucosa with mild chronic gastritis; H. pylori not identified. Final Diagnosis: Mild chronic gastritis; negative for H. pylori.	
Pathologist: Dr. Clara Ng Report Date/Time: 03/16/2023, 11:00 AM	

After Visit Summary / Discharge Instructions

Recovery Course: The patient was monitored in recovery with stable vitals. Reports no pain or nausea, and tolerated sips of water well.

Diet/Activity Instructions: Clear liquids today, advance to normal diet as tolerated. Avoid strenuous activities for 24 hours.

Warning Signs: Contact if experiencing severe pain, fever, or large amounts of bleeding.

Follow-Up Plan: Continue omeprazole and follow up with primary care physician in 2 weeks.

Medication Plan: Continue all home medications. Resume aspirin tomorrow morning.

Results Communication Plan: You will be contacted with pathology results within one week.

Next Appointment Window: Scheduling office will contact you by phone.

Patient Understanding Statement: Verbally confirmed understanding of instructions.

Escort/Transport Note: Discharged to home in the company of spouse.

Discharge Educator: Susan White, RN | 10:45 AM

Provider Signature Block

Dr. John Doe, MD

Date/Time: 03/15/2023, 10:30 AM

Electronically signed by Dr. John Doe

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